

Generalized Anxiety Disorder: Patient Information Leaflet

Excessive, hard-to-control worry about everyday events, lasting months, with physical symptoms like restlessness, muscle tension and poor sleep.

What is Generalized Anxiety Disorder (GAD)?

Generalized Anxiety Disorder (GAD) is one of the most common psychiatric conditions seen in both general practice and psychiatric clinics. It is characterized by persistent, excessive worry about multiple areas of daily life — work, health, family, finances, or routine responsibilities — that is disproportionate to the actual likelihood or impact of the feared events. Unlike everyday stress, which passes once a specific problem resolves, GAD worry tends to be free-floating, shifting from one concern to the next, and is accompanied by physical symptoms that can significantly affect sleep, concentration, relationships and work performance.

Common symptoms

- Excessive worry about multiple things (work, health, family, finances) most days for 6+ months
- Feeling restless, keyed up, or on edge
- Becoming easily fatigued
- Difficulty concentrating or the mind 'going blank'
- Irritability
- Muscle tension, headaches, or unexplained body aches
- Sleep disturbance — difficulty falling asleep, staying asleep, or unsatisfying sleep
- Physical symptoms such as a racing heart, sweating, trembling, or gastrointestinal discomfort during periods of high anxiety

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GAD develops through an interaction of genetic vulnerability, neurobiological factors and life experience. First-degree relatives of people with GAD have a higher risk of developing an anxiety disorder, suggesting a heritable component. Early life stress, childhood adversity, overprotective or inconsistent parenting, and a temperament marked by behavioral inhibition (a tendency to be cautious, shy or easily distressed as a child) all increase vulnerability. Ongoing chronic stress — financial pressure, relationship difficulties, illness, or major life transitions — commonly triggers or worsens the disorder in someone who is already predisposed.

Treatment options

GAD is highly treatable. First-line treatment usually combines psychotherapy — particularly Cognitive Behavioral Therapy (CBT) — with medication when symptoms are moderate to severe, or when therapy alone is insufficient. Treatment is individualized based on symptom severity, coexisting conditions, personal preference, and past response to treatment. Most people see meaningful improvement within weeks to a few months of starting appropriate treatment, though building lasting skills to manage worry is an ongoing process.

Self-help tips

- Maintain a regular sleep schedule and prioritize 7–9 hours of sleep
- Reduce or eliminate caffeine, especially in the afternoon and evening
- Engage in regular physical activity — even brisk walking most days measurably reduces anxiety
- Practice structured relaxation or breathing exercises daily, not just during high-anxiety moments
- Limit alcohol, which disrupts sleep and can worsen anxiety as it wears off
- Set aside a specific, time-limited 'worry period' each day rather than worrying throughout the day

Prognosis

With appropriate treatment, most people with GAD experience significant symptom

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fluctuating course — symptoms may wax and wane with life stress — but ongoing use of learned coping skills, and continuation or resumption of treatment during high-stress periods, allows most patients to lead full, productive lives. Untreated, GAD tends to persist and is associated with higher rates of depression, other anxiety disorders and reduced quality of life.

When to seek urgent help

Seek urgent psychiatric or medical attention if anxiety is accompanied by thoughts of self-harm or suicide, chest pain, or if you are using alcohol or other substances to cope with anxiety. Seek same-day medical review if physical symptoms (palpitations, breathlessness) are new, severe, or you are unsure whether they are anxiety-related or a separate medical emergency — anxiety should not be assumed to explain new physical symptoms without appropriate evaluation.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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